Getting Mum **to her appointment.**

One place to arrange an older relative's medical appointment and the ride to it — and live proof, minute by minute, that it is actually happening.

DOCUMENT	PREPARED	AUDIENCE
Concept, features, status, market and revenue	15 September 2026	Non-technical review

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SECTION ONE

In brief

> CareBridge is a coordination product for families arranging medical appointments and non-emergency transport for an older relative — and for the transport companies that carry them.

An adult child, often living in another city, needs to get a parent to a clinic appointment. Today that means a sequence of phone calls to parties who do not talk to each other: the clinic, a transport operator, a driver whose name nobody knows, and a parent who does not hear her phone ring in her bag. CareBridge puts the appointment, the ride, the live position of the car and the payment in one place, and keeps everyone who should know informed without anybody having to ask.

The distinctive feature is **live tracking that is honest about itself**. While a journey is under way the family sees the car moving and an arrival time counting down. If the position stops updating, the product says so plainly rather than continuing to draw a confident-looking car in a plausible place. That single commitment shapes a great deal of the rest of the design, and it is the reason a family stops phoning a dispatch line — which is, in turn, the reason a transport operator agrees to use it.

4	2	~85%	0
Distinct users served — family member, patient, driver, dispatcher	Paying sides — households and transport operators	Of the planned product is built and demonstrable end to end	Real journeys carried so far — the pilot has not begun

Where it stands

The product works. A complete journey — booking, dispatching, assigning a driver, watching the car arrive, the appointment, the return leg, the receipt — runs today from end to end, with everything stored permanently and nothing reset between sessions. Four surfaces exist and are usable: the family app, the driver app, the dispatcher's console, and a back office.

What has *not* happened is the part that cannot happen at a desk. No real car has driven a real route with a real phone in the cradle. No live map provider, card processor or email service has been connected to a live account — each is written and waiting for the account. And one feature required by both app stores, the ability for a user to delete their own account, is designed but not yet built. Section eight sets all of this out honestly and section ten puts it in order.

> **A NOTE ON WHAT THIS PRODUCT DELIBERATELY IS NOT**
>
> CareBridge holds no diagnoses, no medications, no clinical notes and no lab results, and it offers no advice of any kind. It records that an appointment exists, at which clinic and at what time, and it arranges the car. I made that boundary deliberately and early, for reasons I set out in section seven — but you are far better placed than I am to judge whether the line falls in the right place, and I would genuinely welcome your view on it.

SECTION TWO

The problem being solved

> The failure here is informational, not clinical. Every party holds one fragment of the truth, and none of them holds the whole picture.

The call

An adult child, two states away, gets a call: Mum has a cardiology follow-up on Thursday at 10:40. She no longer drives. What follows is a logistics problem that no single system owns.

- Call the clinic to confirm the slot, and how long it is likely to run.
- Call a transport company. Explain the walker, the wheelchair, the fact that she cannot manage stairs unassisted.
- Book the return leg without knowing when the appointment will actually end.
- On Thursday morning, wait. Was she collected? Did the driver arrive at all? Did she get inside the building? Call her mobile; she does not hear it. Call the transport company's dispatch line and hold.
- Pay three separate parties, none of whom talk to each other.

Why it is hard

PARTY	KNOWS	DOES NOT KNOW
Clinic	The slot, roughly how long it will run	Whether the patient is on the way
Transport operator	Which driver, which vehicle	When the appointment will end
Driver	Where the car is, right now	Anything about the appointment
Family	That an appointment exists	Essentially everything else, on the day

Who feels it

The family member feels the anxiety and is willing to pay to remove it. The patient feels the disruption — a stranger at the door with no name attached, a car she was not told about. The dispatcher feels the phone calls, which are pure cost to their business. And the clinic, I imagine, feels the no-shows, though you would know better than I would how much of a clinic's no-show rate is genuinely a transport problem rather than something else.

> **THE THING THAT ACTUALLY DRIVES PEOPLE AWAY**
>
> It is not a missed appointment. It is **the two hours of silence in the middle of a workday** — the interval between "the car should have arrived by now" and someone finally picking up a phone. That interval is what CareBridge is built to remove, and the whole product follows from treating it as the real problem.

What "solved" looks like

A family member opens one app and can answer three questions without having to interact with it at all:

- What is next?
- Is it on track?
- What do I do if it is not?

Everything else in this product exists to make those three answers true and current.

What it does not try to solve

Not the clinical problem. Not the insurance problem. Not the shortage of accessible vehicles. Coordination is the fragment nobody owns, and the one where a small team can be decisively better than the alternative — which today is a telephone and a paper diary.

SECTION THREE

What CareBridge does

> Six things, each of which exists to keep one of those three questions answered.

CAPABILITY	WHAT IT MEANS IN PRACTICE
The care circle	A family member creates a record for their relative and invites a sibling, a neighbour or a paid carer in. Everyone in the circle sees the same schedule and receives the same alerts — not just whoever happened to make the booking. Each invitation is single-use, expires, and is tied to one verified email address, because it grants standing access to a vulnerable person's home address and daily movements.
Appointments	The appointment is recorded with its clinic, date, time and expected duration, and reminders are scheduled against local wall-clock time, so "the day before at 10:40" survives a daylight-saving change. To be precise about the limit: the appointment is *recorded* in CareBridge, not booked into a clinic's own scheduling system.
Transport, including round trips	A round trip is treated as two separate journeys, not one journey with two halves, because nobody knows when an appointment will end. Each leg is assigned, tracked, priced and cancelled on its own. The family sees an itemised price estimate before confirming anything.
Dispatch and assignment	A dispatcher at the transport company sees a queue ordered by when each car is actually needed, and assigns a driver and vehicle. Access needs — a wheelchair, for instance — are enforced as hard constraints, not suggestions.
Live tracking	While a journey runs, the family watches the car on a map with an arrival time that counts down, and gets a notification at each step: on the way, arrived, picked up, in progress, arrived at the clinic, done. If the position goes stale the app says so, and past two minutes it stops showing a position at all.
Money and the record	The fare is quoted up front, authorised when a driver is assigned, and charged on completion. Afterwards there is a receipt and a complete timeline of what happened and when — which doubles as the record both sides reach for when they disagree about a late pickup or a no-show.

The one thing that makes it a product rather than a tool

A live map is the only feature that turns an administrative product into an emotional one, and emotional products retain customers. Concretely it does four things a list of statuses cannot:

- **It replaces a phone call.** Every avoided call to dispatch is a cost saved on both sides of the transaction.
- **It bounds anxiety.** "Driver is six minutes away" is a finite, tolerable wait. "No update since 09:12" is not.
- **It produces evidence.** No-show disputes, late-pickup complaints and refund requests are all settled by the same timeline.
- **It is the way in to the transport operator.** Operators adopt CareBridge because *their* customers stop calling them — not because they wanted new software.

SECTION FOUR

The four people it serves

> The family member pays, the patient benefits, the driver supplies the information that makes the family's side worth paying for, and the dispatcher decides whether the operator renews. All four must be served or the loop does not close.

Ada

The family member — 45 to 65, working, often two states away. The buyer, and the daily user.

Needs: certainty, fewer phone calls, proof of what happened.

She uses the app between meetings, on a phone, with about thirty seconds of attention. She opens it to answer one question and expects it answered at the top of the screen. What loses her: a map that looks confident and is wrong; a notification that says nothing and links nowhere; being told to call dispatch.

Margaret

The patient — 70 or older. May have low vision, a tremor, hearing loss, or mild cognitive impairment.

Needs: to know who is coming, when, and in what car. Nothing else.

We assume she frequently will not install an app at all, so the family app must work completely with her never touching it. Where she does use it, there is a simplified mode with large touch targets, larger type and high contrast — a mode inside the family app rather than a third separate app to install. What loses her: density, jargon, and anything that requires remembering a previous screen.

Tunde

The driver — contract or employed, phone mounted, gloves on, running late.

Needs: the next task, one tap, no reading.

He works in a moving vehicle, in sunlight, with patchy signal. So the button that moves a job forward is the largest thing on the screen, every tap is stored and re-sent if he is in a dead zone, and the app shows a permanent notice while it is sharing position so he can always see that it is on. What loses him: battery drain, a tap that silently fails, and being asked to type.

Priya

The dispatcher — operations staff at a transport company, 40 to 200 journeys a day.

Needs: density, exceptions surfaced, fast reassignment.

She is at a desk with a large screen and a keyboard all day. This is the one place where packing in information beats simplicity, which is why her console is a desktop screen and not a phone. What loses her: a queue that does not tell her which journey is about to become a problem.

WHY THIS SPLIT MATTERS COMMERCIALLY

A decision that improves one of these four at the cost of another breaks the loop. The clearest example is battery drain on the driver app: if it flattens a driver's phone by four in the afternoon, he turns it off, the family's map goes dark, and both subscriptions are at risk. That makes it a revenue problem, not merely a technical one — which is why it has a measured threshold attached to it in section thirteen.

SECTION FIVE

How a journey actually runs

> Six stages, each of which has a defined "done". This is the sequence a real family would move through.

1. **Getting set up**

   Ada registers, confirms her email, creates a record for Margaret, notes her mobility needs — walker, escort to the door — and an emergency contact, then invites her brother into the care circle. He sees Margaret in his own app with exactly the permissions Ada chose, and if she later revokes them, they disappear immediately.

2. **Recording the appointment**

   The cardiology follow-up goes in: clinic, Thursday 10:40, expected 45 minutes. It appears for everyone in the circle, and reminders are set against the clinic's own local time rather than Ada's.

3. **Requesting transport**

   Ada requests a round trip against that appointment and sees an itemised price estimate before confirming. Two journeys are created. The return leg can be marked flexible, because in practice nobody knows when the appointment will finish. Cancelling the appointment cancels both.

4. **Assignment**

   The transport company's dispatcher assigns a driver and vehicle from a queue ordered by when each car is needed. Ada is notified and can see the driver's first name, photo and vehicle. Payment is authorised at this point rather than at the end. A wheelchair journey cannot be given to a vehicle that cannot carry one, an unapproved driver cannot be assigned at all, and no driver can carry two passengers at once.

5. **The journey**

   The driver moves through a fixed sequence — on the way, arrived, passenger on board, in progress, arrived at the clinic, completed — one tap at a time. Ada watches the map and gets a notification at each step. If the driver is held up, a delay can be raised and then cleared without losing where the journey had got to. Position sharing stops the moment the journey ends, and is refused after that.

6. **Afterwards**

   The fare is charged, a receipt is available, and everything lands in a timeline Ada can read back weeks later. The timeline is complete even if a notification failed to reach somebody's phone, because it is built from the record of what happened rather than from the messages about it.

THE JOURNEY NOBODY DESIGNS FOR

Margaret's phone is in her bag and she does not hear it. Ada is in a meeting. The driver is nine minutes late because of an accident on the bridge. Every one of the six stages above has to still be readable when *that* is what is happening. It is the main reason a delay is treated as a flag raised over a journey rather than as a status the journey moves into — the journey has to be able to return to where it was.

SECTION SIX

The four places people use it

SURFACE	WHO USES IT	WHAT IT IS FOR
The family app	Ada, and everyone in the care circle	The schedule, the booking, the live map, the notifications, the receipts and the household's own plan and card. Contains a simplified mode for the patient, so she is never asked to install anything separate.
The driver app	Tunde, issued by his employer	Today's jobs in the order he will do them, the one-tap sequence, and document upload for his licence and insurance. It is a separate app on purpose — its position-sharing permission should never appear on a family's phone. There is no public sign-up; an operator issues the account.
The dispatch console	Priya, at a desk	The driver roster, shifts, the queue ordered by urgency, assignment and reassignment with a recorded reason, and the operator's own subscription, card and invoices. Dense and keyboard-driven by design.
The back office	CareBridge's own staff	An operational dashboard, a searchable record of every significant action taken in the system, controls for switching features on and off for a percentage of users, and refunds. Reaching it requires both a staff role and a confirmed second factor on the account — no exceptions.

> **WHY THE DRIVER APP CANNOT BE A WEB PAGE**
>
> It has to keep reporting position with the screen off and another app in front of it — a driver is going to be running a navigation app on the same phone. No web browser will do that, so the driver app is the one surface that must be installed on a real device. That is also why it is the one surface that cannot be fully proven without a car, which section ten returns to.

Two more things that exist but are switched off

A **caregiver marketplace** — booking a paid carer for a visit, with commission and cancellation windows — and a **clinic portal**, where a front desk could see expected arrival times, are both built behind the scenes and deliberately switched off. Neither has a screen yet, and my view is that neither should get one until a pilot gives a reason. I would be particularly interested in your read on the clinic portal: whether a front desk would actually use something like that, or whether it would be one more screen nobody logs into, is a question about how clinics really run that I am not well placed to answer.

SECTION SEVEN

The principles that shape it

> A handful of commitments made early, each of which closed off options later. They are worth understanding because they explain most of the product's shape.

Tracking must be honest

A stale position drawn as a confident moving car is worse than no map at all, because it manufactures false certainty about a vulnerable person. So every place a position appears carries its age; accuracy degradation is shown rather than smoothed over; and the product says "last seen three minutes ago" rather than guessing at a plausible position in between. Past roughly forty-five seconds the screen says the reading is ageing; past two minutes it stops drawing a car at all. The same limits are applied when a reading arrives, so this is not a promise about how one app behaves — a position that turns up already expired is refused outright.

Nothing important is decided on a phone

Every permission check, every step of a journey and every notification is decided centrally, not by the app in someone's hand. The apps can grey out a button, but they cannot grant anybody anything. A stolen phone, a tampered app or a stale screen changes nothing about what is actually permitted.

Access is granted to a person, not implied by a link

Knowing a journey's reference is not permission to watch it. Access is resolved every single time, up from the journey to the patient to the specific grant that person holds. And "we could not find that" and "you are not allowed to see that" are the same response word for word, so neither can be used to fish for the other.

Notifications carry no detail

No name, no clinic, no address, no appointment time appears in a notification. A phone face-up on a kitchen table is readable by whoever is in the room. The alert says something has happened and the app says what.

Collect as little as possible

There is no date of birth anywhere in the system. Name, address and date of birth together are the standard set for both identity verification and re-identification, and nothing about arranging a car needs it — an optional coarse age band covers genuine mobility planning. Legal name is optional; preferred name is required, because somebody has to be greeted by the name they actually use. Mobility requirements are recorded operationally — wheelchair, transfer assistance, escort to door — and never as anything diagnostic.

A wheelchair is a constraint, not a preference

A dispatcher under pressure at eight in the morning should not be the last line of defence against a patient in a wheelchair meeting the wrong vehicle at the kerb. The system refuses those assignments outright rather than warning about them.

A failed payment does not blank the map

If a card is declined at renewal, the subscription keeps working through a grace window — a week for a family, a fortnight for an operator — while the problem is chased by email. The obvious alternative cuts tracking off the moment a card expires, and the family's first signal that anything is wrong is an empty screen while their mother is in a stranger's car. Cards expire; it happens to everybody, and disproportionately to the people juggling an ageing parent's logistics around everything else.

> **ON THE CLINICAL BOUNDARY — MY REASONING, OFFERED FOR YOUR CORRECTION**
>
> CareBridge is not a health record, not a triage or advice system, not a medication tool, and explicitly not an emergency service. My reasoning was twofold. First, coordination data — a name, an address, a phone number, an appointment time, mobility needs — is sensitive but bounded; the moment the system stores a diagnosis or a medication it becomes something quite different, with a correspondingly different set of obligations, and I would rather take that step deliberately and late than arrive there by accident. Second, as far as I can tell nothing in any of the six journeys above actually requires knowing *why* someone is seeing a cardiologist.
>
> That second claim is the one I would most like you to test. If there is a category of patient for whom the transport arrangement genuinely depends on clinical detail that this system refuses to hold, I would rather hear it from you now than discover it in a pilot.

One more, about language

The system is described as having a privacy-ready design, and never as "compliant" with any particular regime. Compliance depends on legal determinations, executed agreements, documented safeguards, staff training and an incident process — none of which are properties of a piece of software. I have kept that distinction rigid throughout, and I would rather under-claim to you here than have a claim of mine turn into a problem for you later.

SECTION EIGHT

Where the project stands today

> The honest inventory. Everything marked "working" can be demonstrated end to end right now; everything else is named rather than glossed over.

Working, and demonstrable today

WORKING	The complete family journey — sign up, care circle, invitations, appointments, round trips, live tracking, notifications, receipts
WORKING	Dispatch end to end — driver roster and lifecycle from invitation through approval, suspension and offboarding; shifts; the urgency-ordered queue; assignment and reassignment with recorded reasons
WORKING	The driver app — today's jobs, the one-tap sequence, a queue of taps and positions that survives a tunnel, and document upload
WORKING	The back office — operational dashboard, searchable activity record, feature switches, refunds; reachable only with a staff role and a confirmed second factor
WORKING	Both subscriptions — a household manages its plan, card and invoices in the app; an operator manages its own, priced by drivers on the road, in the console
WORKING	Account security in full — email verification, password reset, a list of active sessions with the ability to revoke any of them, sign-out-everywhere, and app-based two-factor authentication
WORKING	Fares priced from editable rules rather than fixed numbers, itemised, versioned, and calculated identically on every screen that shows them
WORKING	Everything is stored permanently. Nothing resets. A journey continues running even if the family closes the app and reopens it on another device.

Working, but standing in for something real

STAND-IN	**Maps and routing.** The map provider is connected but has never been called against a live account, so the route view is a schematic by design rather than a real street map.
STAND-IN	**Card payments.** Money moves correctly through the whole system, but against a local simulation rather than a real processor account. The real path is written and has not been exercised.
STAND-IN	**Email and push notifications.** Both work; both point at local test services rather than live vendor accounts.
STAND-IN	**Real GPS in traffic.** Everything about the driver app's behaviour on the road is designed for and tested in a simulator. The field test is written and has not been run.
STAND-IN	**Automatic matching.** Assignment is a human dispatcher choosing from an ordered queue. That is deliberate for a pilot, not a shortfall — but it is not automatic dispatch.

Not built

NOT BUILT	**Account deletion.** Designed in full detail, not implemented. Both app stores require it, and Play requires it at a public web address as well. *This is the single item blocking store submission.*
NOT BUILT	**Screens for the caregiver marketplace and the clinic portal.** Both exist behind the scenes and are switched off on purpose, pending pilot evidence.
NOT BUILT	**A live hosted environment.** The recipe for one is written and checked automatically, but it has never been run — which needs a cloud account rather than more work.
NOT BUILT	**A rehearsed restore from backup.** The plan exists; nothing has ever actually been restored. A backup nobody has restored is not a backup.
NOT BUILT	**The pilot paperwork** — a privacy policy, a consent model that covers the patient and not only the family member, and a decision about who answers the phone at eight on a Monday morning.

SECTION NINE

Performance and reliability

> What the system is built to sustain, and the specific promises it makes about its own honesty.

Scale

The pilot is sized for one transport operator in one metropolitan area, carrying under roughly 500 journeys a day. At that volume the whole system runs comfortably on a single modest setup with nothing exotic in it. That is a deliberate choice rather than a limit discovered late: building for volume the pilot will not have would have cost months and proved nothing. The route from there to substantially more volume is understood and does not require starting again.

PROMISE	TARGET	WHY THAT NUMBER
Position freshness on screen	under 30 seconds	Beyond about two minutes, a map is lying by omission — so that is where it stops drawing a car
Position updates while moving	every 5 seconds	Sub-second tracking is not needed here and would cost battery for no benefit
Driver battery over a four-hour shift	under 15%	Above 25% a driver will simply turn it off, and the family's map goes dark
Taps lost to bad signal	zero	Every tap is stored on the phone and re-sent when signal returns
Position sharing after a journey ends	zero	Sharing stops within seconds of completion, and later readings are refused outright
Time from request to assignment	under 15 minutes	Beyond an hour, the dispatcher's screen is not doing its job

Things that are treated as incidents rather than targets

Four have no acceptable number. Any single occurrence is investigated:

- Personal information appearing in a diagnostic record.
- A location visible to somebody without a grant.
- An action that succeeded without being written to the activity record.
- A notification containing a patient name, clinic name, address or appointment time.

The first three are checked automatically on every change made to the system. The fourth is checked against the actual text of real notifications, not against a template that could drift away from what is really sent.

Failures the system anticipates

SITUATION	WHAT HAPPENS
Driver enters a tunnel	Taps and positions queue on the phone and reconcile when signal returns. The family sees the position ageing and then being withdrawn, rather than a car frozen at a junction.
Driver's phone dies	Silence is the one failure nothing else can detect — a stationary car looks like a car at traffic lights. A watchdog raises it to the dispatcher, who is the person who can pick up a phone.
Family taps "book" twice, or the connection drops mid-request	One car is booked, not two. A retried request is answered from the original record rather than performed again.
Someone's access is revoked mid-journey	Their live view is cut within about fifteen seconds. Access is re-checked continuously for anyone watching, not once at the start.
A payment notification arrives twice	It is recorded once. Duplicates are recognised and discarded.
A card is declined at renewal	Service continues through the grace window while the problem is chased by email. See section seven.

SECTION TEN

What must become real before a pilot

> In the order the items block each other. Two of them are pure waiting and can be started immediately.

	ITEM	WHAT IT NEEDS, AND WHAT IT COSTS TO LEAVE UNDONE
1	**A hosted environment**	A cloud account and a modest monthly spend. Until one exists there is nothing for a phone to talk to — a phone is the one device that cannot be pointed at a laptop. *Blocks everything below.*
2	**Vendor accounts**	Email, maps, push notifications, card payments. Each is written and waiting. Without email, nobody can sign up at all. Without payments, a pilot can run free but not paid. The email provider starts in a restricted mode that takes several days to be released from — worth starting first.
3	**Devices**	One iPhone, two Android phones — one of them deliberately cheap and old, because that is where aggressive battery management lives — and a car.
4	**The field test**	Written, ready to run, never run. It measures the two things currently designed for rather than proven: battery drain over a real shift, and whether real dead zones are shorter than the offline queue. My strong view is that it happens *before* the pilot, not during it — a pilot that discovers the battery claim is wrong discovers it using somebody's mother.
5	**Account deletion**	The one remaining build item that blocks store submission. The design work is done; building it is mechanical rather than a series of judgment calls.
6	**Store enrolment**	Apple's organisational enrolment takes days to weeks. Pure waiting, and it can start today.
7	**Consent and a privacy policy**	The system holds home addresses, appointment times and live location about people who are usually *not* the account holder. A consent model that asks only the daughter is not, I think, meaningful consent — and this is one place where I would very much want your judgment before anything is finalised.
8	**A rehearsed restore**	Finding out during a pilot that a backup does not restore means finding it out with somebody's real data.
9	**Who answers the phone**	A driver is late, a family is worried, and an app is not a person. Worth deciding before the pilot rather than discovering at eight on the first Monday.

> **THE TWO LONG POLES**
>
> Store enrolment and the email provider's release from its restricted mode are both measured in days to weeks, both are pure waiting, and both can be started before a single other item on this list is touched. If nothing else from this document is acted on immediately, those two are worth starting.

SECTION ELEVEN

The market and the opening

> Three categories of product already exist near this problem. Each solves a fragment and none solves the join — which is precisely where the opening is.

WHAT EXISTS TODAY	WHAT IT DOES	WHAT IT LEAVES UNDONE
Non-emergency medical transport brokers	Move people, at scale, including in accessible vehicles	Tell families essentially nothing. The dispatch phone line *is* the interface.
Consumer ride-hailing	Tracks beautifully. Everyone already understands the map.	Cannot handle a wheelchair, a scheduled pickup booked days ahead, or a passenger who needs an arm to lean on.
Caregiver marketplaces	Staff visits, vet carers, handle scheduling of people	Ignore transport entirely.

The defensible position is the **join** between those three, held together by a complete and trustworthy record of what happened. Owning the link between appointment, journey and payment makes caregiver, clinic and insurance layers natural extensions later. Starting at any of those layers instead makes coordination a retrofit — which is much harder.

The constraint we accepted knowingly

CareBridge takes payment by card only. It does not bill insurance, Medicaid or Medicare. This narrows the addressable market substantially — a large share of non-emergency medical transport volume is publicly funded — and it was accepted on purpose for the pilot, on the reasoning that building payer integration before proving the coordination value would mean building the hardest part first for a product nobody has yet said they want.

> **WHERE I WOULD MOST VALUE YOUR JUDGMENT**
>
> This constraint is the single largest open question in the whole plan, and it is one where your experience is worth considerably more than my analysis. My assumption is that there is a meaningful segment of families who pay privately for transport already — because a parent is not eligible, because the eligible option is unreliable, or simply because they would rather not deal with the paperwork — and that this segment is where a product like this starts. If that assumption is wrong, or much smaller than I think, the plan should change shape early rather than late. It is deliberately structured so that opening up publicly-funded billing later changes the size of the market without requiring the product to be rebuilt.

How it would be sold

Transport operators adopt CareBridge because *their* customers stop calling their dispatch line — not because they wanted new software. That is the entire sales argument, and it is why the dispatcher's console being genuinely good matters commercially: the dispatcher is the person who decides whether the operator renews. Families are reached through the operator they are already using, rather than acquired independently, which keeps early customer acquisition cost close to nil.

SECTION TWELVE

How it makes money

> Two payers, one mechanism. This is the decision most of the commercial plan hangs off, and it changed once already for a reason worth knowing.

What changed, and why

The original model had one payer: a family paid a flat fee *and* the fare for each journey, with the margin taken as a percentage of that fare. That had a structural problem. It made revenue rise exactly when a transport operator's own margin was thinnest, and it gave the operator a reason to route their busiest days around us. The operator was also the party whose adoption decided whether the product existed at all — and the one party we had no commercial relationship with whatsoever. So both sides now subscribe.

	THE FAMILY	THE TRANSPORT OPERATOR
What they buy	Coordination — booking, live tracking, reminders, the care circle	Operations — the dispatch console, the driver app, assignment
Priced by	Household	**Drivers on the road**
Cadence	Monthly or annual	Monthly or annual
Also pays	The fare for each journey	—

Why the operator is priced per driver

It is the only number an operator already tracks, and the only one that moves with the value they get. Charging per journey was the alternative and is worse for both sides for the reasons above. Charging per dispatcher was also considered and rejected: a company with three dispatchers and forty drivers gets forty drivers' worth of value.

The ladder is **graduated** — the first few drivers are included in the base price, the next band is priced one way, and larger fleets another — so an operator's bill never *falls* when they hire someone. The alternative, where the whole fleet re-prices at the rate the total reaches, produces exactly that, and explaining it ends in a spreadsheet nobody trusts again.

The fare, and who takes a cut of it

- **Operator on a subscription** → the whole fare passes through to them. The margin is their seats, taken a month earlier. Charging a percentage as well would be charging twice for one relationship, which an operator finds in a spreadsheet six months in and never forgives.
- **Operator not yet on a subscription** → a platform fee applies to the fare. This exists so a pilot operator can start before they have signed anything, not as a second revenue line.

The family pays the same total either way. That is deliberate: a price that moved with our commercial arrangements would be a price the family could not predict. Which way each journey settled is recorded against it, so a payment to an operator can still be explained months later.

The fairness rules around seats

Adding a driver is charged immediately and pro-rated. Removing one takes effect at the next renewal and is not refunded — the seat stays usable until the period that paid for it ends. The asymmetry is stated on the pricing page rather than discovered on an invoice. What it removes is the option to churn seats daily around a renewal date; what it costs an operator who genuinely shrinks is at most one period. A driver suspended and reinstated within the same period is not charged twice.

Cost structure

COST	WHAT DRIVES IT
Transport itself	Partner operators. CareBridge owns no vehicles and employs no drivers. This is contractual, not a cost line that scales with us.
Maps and routing	Scales with *tracked journeys*, not with users. Arrival-time lookups are throttled and cached, and spending is alarmed. This is the one line that could threaten the unit economics, so it has a stop condition attached in section thirteen.
Infrastructure	Roughly flat at pilot scale. Nothing exotic is added without a stated need.
Card processing	A percentage, on both payers.
Support	A human inbox, business hours, during the pilot. Every avoided dispatch call is the product working.

What would change the model

Four things, each measured in the pilot rather than guessed at now: families who will not pay a subscription but will pay a per-journey premium; an operator who refuses per-driver pricing; operators subscribing while families do not (in which case the dispatch console becomes the product that matters most); and access to publicly-funded transport billing, which changes the shape of the market entirely. The underlying mechanism handles all four — what changes is the numbers in a table, not the product.

SECTION THIRTEEN

How we will know it is working

> Every measure below has a stop condition — a number that, if we hit it, means something is wrong with the idea rather than with the execution. A measure without one is decoration.

The one that matters

CALLS TO DISPATCH, PER COMPLETED JOURNEY

This is the product's entire claim in a single number. If a family can watch the journey, they do not phone to ask about it — and the operator's dispatch line gets quieter, which is why the operator renews.

Baseline: measured with the pilot operator before launch. **Target:** a 50% reduction within two months of a family's first tracked journey. **Stop condition:** no measurable reduction after 100 tracked journeys. If families still call, the map is not doing the job the whole product is built around.

Is the product worth opening?

MEASURE	TARGET	STOP CONDITION
Families active weekly, where an appointment is coming up	above 70%	below 40%
Time from opening the app to knowing "is it on track"	under 5 seconds	any tapping required
Journeys tracked end to end, of those assigned	above 90%	below 70%
Appointments with transport requested	above 60%	below 30% — we are a calendar, not a coordination product

Is the promise about honesty being kept?

MEASURE	TARGET	STOP CONDITION
How stale a shown position ever gets	under 30 seconds	over 2 minutes
Driver battery over a four-hour shift	under 15%	over 25%
Taps lost to connectivity	zero	any

Does the business work?

MEASURE	TARGET	STOP CONDITION
Households converting from trial	above 25%	below 10% — families will not pay for this
Monthly churn	under 5%	over 12%
Map and routing spend per tracked journey	under $0.15	over $0.50 — the unit economics do not work
Payments succeeding first time	above 95%	below 85%
Time from request to assignment	under 15 min	over 1 hour
No-show rate	under 3%	over 8%
Pickup within 10 minutes of schedule	above 85%	below 60%

> **TWO OF THOSE NUMBERS I WOULD LIKE YOU TO SANITY-CHECK**
>
> The no-show and on-time targets are the two I am least confident in, because they are the two that depend on how clinics and patients actually behave rather than on how the software performs. I took them from what seemed reasonable rather than from operational experience. If a 3% no-show target is naive, or if pickup within ten minutes of schedule is either trivially easy or wildly optimistic in practice, that is worth correcting before we set it as a bar with a pilot operator.

Growth rate and anything that depends on volume the pilot will not have are deliberately not measured. The pilot's job is to test the idea cheaply, not to look like traction.

SECTION FOURTEEN

Risks, stated plainly

RISK	WHAT HAS BEEN DONE ABOUT IT
Families will not pay a subscription	The largest commercial unknown. Mitigated by pricing the operator separately, so the business is not wholly dependent on the household line, and by the trial conversion measure above, which is designed to fail fast rather than slowly.
Battery drain drives drivers to switch it off	Sampling backs off as the battery falls, and the app shows a permanent notice so a driver always knows it is running. But the claim is designed for rather than proven, which is precisely what the field test exists to settle — and it has not been run.
Map and routing costs exceed the value of a journey	Arrival-time lookups are throttled and cached rather than requested continuously, and the spend has an alarm and a stop condition attached. The vendor sits behind a boundary so it can be changed without disruption if the pricing turns out wrong.
The dispatcher's console is not good enough for a real operator	This was tracked as a risk until the console shipped, because an operator who finds it worse than their existing whiteboard will not use it. It now exists and is genuinely dense and fast. Whether it is *good enough* is a pilot question.
Card-only narrows the market too far	Accepted knowingly, revisited with pilot evidence. See section eleven, where I have flagged this as the place I would most value your view.
A family segment will not install an app	There is no web version for families in the first release. If the pilot shows a segment that will not install anything, adding one is an unblocked path rather than a rebuild.
Consent that only asks the account holder	Openly unresolved. The system holds sensitive information about people who are usually not the account holder, and getting this right is a matter of judgment rather than engineering. Flagged in section ten as needing your input.
Driver vetting	Worth being explicit: CareBridge *records* a driver's status, licence and insurance documents, and requires approval before that driver can be assigned to anybody. It does not itself verify identity or run background checks — that happens at the transport operator, and the product will never describe itself as if it did otherwise.

SECTION FIFTEEN

What I would suggest next

> The product is built. What remains is almost entirely about moving it from a demonstration to something real people use — and most of that is arrangements rather than construction.

Start immediately, because they are pure waiting

- **Apple's organisational enrolment.** Days to weeks, and nothing else can be submitted until it is done.
- **The email provider's release from its restricted mode.** Several days, and without it nobody can sign up at all.

Then, in order

- **Stand up a hosted environment** and connect the four vendor accounts. This unblocks everything else and is largely a matter of opening accounts.
- **Build account deletion.** The one remaining piece of construction that blocks both app stores. Fully designed already.
- **Run the field test** — one car, one route, one real driver, half a day. This settles the battery and coverage claims, which are the two things currently taken on trust.
- **Settle the paperwork** — privacy policy, consent model, and who answers the phone.
- **Find the pilot operator.** One transport company, one city. Measure their dispatch call volume *before* anything is switched on, because without that baseline the most important number in the plan cannot be calculated afterwards.

Three questions I would like to put to you

- Is the clinical boundary in the right place — is there genuinely nothing about arranging transport that requires knowing why someone is seeing a specialist? (Section seven.)
- How large, in your experience, is the segment of families who already pay privately for medical transport? This is the assumption the whole commercial plan rests on. (Section eleven.)
- Would a clinic front desk actually use a portal showing expected arrival times, or is that one more screen nobody logs into? It is built and switched off, so the answer costs nothing to act on either way. (Section six.)

In one sentence

CareBridge works today, end to end, and the gap between here and a real pilot is mostly accounts, arrangements and one afternoon in a car — not more building. The main open questions are commercial rather than technical, and the three above are the ones where your judgment would change what we do next.

CareBridge coordinates medical appointments and transport. It holds no medical records, offers no medical advice, and is not an emergency service. Nothing in this document should be read as a claim of compliance with any particular regulatory regime; the system has been designed with those requirements in view, but compliance itself depends on legal determinations, executed agreements and operational practices that sit outside the product.